

Copy to Patient

MRI - MRI Shoulder Arthrogram - Left

MRI ARTHROGRAM LEFT SHOULDER

TECHNIQUE:

Arthrogram protocol

FINDINGS:

AC JOINT:

The AC joint is maintained. Patient has a persistent os acromiale with some degenerative cystic changes along the margins suggesting there is some added movement at this level. This can be seen in association with impingement.

ROTATOR CUFF:

The teres minor muscle and tendon appear intact. There is some mild thinning of the distal insertional fibers superiorly of the subscapularis keeping with a partial tear at this level. The bulk of the subscapularis is intact. The supraspinatus and infraspinatus tendons appear intact.

BICEPS TENDON AND LABRUM:

The tendon of the long head of the biceps is completely torn at this level of the bicipital labral complex and is retracted distally approximately 3 to 4 cm from the top of the greater tuberosity. The bicipital labral complex is elevated off the glenoid and appears to be quite frayed consistent with the tear of the long head of the biceps at this level. Anterior labrum is elevated off the glenoid as well which appears to be slightly more prominent than a typical sulcus. This is considered suspicious for anterior superior labral tear. While there is slightly increased signal intensity in the inferior labrum at the 5-6 clock position no discrete contrast is seen undercutting the labrum.

BONE AND GLENOHUMERAL JOINT:

There is some increased signal identified at the posterior aspect of the humeral head. But no definite Hill-Sachs injury is identified. The marrow signal otherwise is within normal. The glenohumeral joint space is maintained. Articular cartilage is preserved.

The musculature around the shoulder is otherwise unremarkable.

IMPRESSION:

Suggestion of a subtle partial tear at the superior insertional fibers of the subscapularis. The rotator cuff otherwise appears intact.

There is a complete tear of the tendon of the long head of the biceps at the level of the bicipital labral complex with retraction of the biceps tendon distally to the level of the proximal humerus.

There is fraying of the superior and anterior labrum with some elevation of the anterior superior labrum off the glenoid suspicious for a labral tear at this level.

There is some increased signal posterior humeral head which may represent a contusion or represent some chronic cystic change from repetitive injury. Additionally there is some mild degenerative changes at the pseudoarticulation of the os acromiale which should be suggestive of some added motion at this level. Again the os acromiale can be associated with impingement.